

::Q1:: A 28-year-old male presented to the ER with a complaint of worsening of body swelling, productive cough and pleuritic type of chest pain of 2 weeks duration. He has noticed bilateral leg swelling that started 3 months ago, which later progressed to involve the abdomen with associated orthopnea and paroxysmal nocturnal dyspnea. Physical exam revealed an acutely sick looking patient on respiratory distress Vital sign: BP 110/60mmHg PR 110 irregularly irregular RR 28 T 37.9 SPO₂ 90% with room air. Chest Decreased air entry with bilateral basal crepitation, CVS raised JVP Murmur of MR, TR. Which of the following is the best immediate management of the patient?

{~ Emergency cardioversion

~ Oxygen support

= Diuretics

~Betablocker}

::Q2:: Which among the following is TRUE regarding infective endocarditis:

{=Etiology after 1 year of valve surgery is as same as native valve endocarditis

~ Fever is present in all patients with infective endocarditis

~ IV drug abusers commonly have culture negative endocarditis

~ Absence of vegetation on transthoracic echocardiography rules out endocarditis}

::Q3:: In a 45-year-old diabetic, male patient presenting with an acute chest pain to the ER of 5 hours duration. Vital sign- Normal. Which is the most immediate step in the diagnosis of this patient?

{~Stress ECG

~ Cardiac angiography

=ECG and cardiac biomarkers

~ Stress echocardiography}

::Q4:: A 58-year-old female diabetic and hypertensive patient presented with a complaint of pain on her right lower extremity upon walking and at rest.

Which of the following is TRUE regarding the diagnosis of this patient?

{~Pain at rest makes peripheral arterial disease unlikely

=An ABI of <0.90 diagnoses peripheral arterial disease

~Doppler ultrasound is gold standard

~Treadmill testing is gold standard }

::Q5:: In the approach of a patient presenting with acute chest pain, which of the following is not true:

{~Reproducible pain by pressure suggests pericarditis

= crescendo pattern of pain suggests unstable angina

~CK-MB is equally sensitive to cardiac troponin

~Management includes oxygen supplementation for all patients}

::Q6:: A 55-year-old female known to have ischemic heart disease, diabetes and hypertension, came for routine follow-up: Current medications include: Metformin, Atrovastatin, Aspirin and Amlodipine and Metoprolol. Recent echocardiography showed IHD with EF 30%. Vital signs are stable and she has no chest pain. Which of the following is the most appropriate medication to be added?

{~Digoxin

~Spironolactone

=Lisinopril

~Isosorbide dinitrate}

::Q7:: A known hypertensive and cardiac patient on Atenolol, Furosemide, Enalapril is admitted with acute decompensated heart failure. On investigation, Chest X- ray showed pulmonary edema and on ECG he has sinus tachycardia. Which one of the following is true in the management of this patient?

{~Continue PO furosemide

=Continue Atenolol

~Change Enalapril to Candesartan

~Add Digoxin}

::Q8:: Which of the following suggests an acute chest pain due to myocardial infarction?

{~Constant and persisting pain

=Pain radiating to both arms

~Reproduceable pain with palpation

~Pleuritic chestpain}

::Q9:: A 69-year-old type 2 diabetic patient was admitted with KILLIP Class I inferior wall STEMI to the medical ICU. The medications he is currently taking are: Asprin, Atrovastatin, Clopidogrel, Metoprolol, Morphine, UFH and Nitroglycerine. His blood pressure dropped the following day. All medications should be held EXCEPT,

{~Morphine

=UFH

~ACE-I

~Nitroglycerine}

::Q10:: A 22-year-old woman from Ambageorgis presents with shortness of breath. You ordered echocardiography and the result turned with severe MS. What will you do about rheumatic fever prophylaxis in this patient?

{~ Prophylaxis for the next 10 years

= Prophylaxis till she turns 40 years

~ No prophylaxis as she is >21 years old

~ Prophylaxis for the next 5 years}

::Q11:: Which of the following is acceptable for rheumatic heart disease prophylaxis?

{= Penicillin G benzathine 1.2 million units intramuscularly every 4 weeks

~ Clindamycin 300 mg po TID

~ Clarithromycine 500 mg PO BID

~Vancomycin 1 gm PO monthly}

::Q12:: A 22-year-old statistics student from Maraki campus presents to the emergency department with 2-h history of rapid heartbeat associated with dizziness. The blood pressure is 90/60 mmHg, ECG is shows atrial fibrillation with ventricular preexcitation. What is the best treatment option?

{~IV metoprolol

~ IV digoxin

=Cardioversion

~ IV amiodarone}

::Q13:: A 31 -year-old lecturer from Tewedros campus who had mechanical valve 4 years ago for severe rheumatic MS, on warfarin, develops paroxysmal atrial fibrillation. He would like to pursue anticoagulation with a novel oral anticoagulant as he read from internet that these drugs are new and more effective. He had also labile INR in the recent month. Which of the following do you recommend?

{~Pradaxa

~ Rivaroxaban

~Apixaban

=continue warfarin}

::Q14:: Which of the following is not a standard indication for anticoagulation in patients with dilated cardiomyopathy?

{= Male, 30 years, DCM, EF of 10%

~ Male, 30 years, DCM, EF of 25%, apical thrombus

~ Male, 30 years,DCM, EF is 20%,multiple transient ischemic attacks

~Male, 30 years, DCM, EF 35%, dyspnea and AF}

::Q15:: A 24-year-old pregnant woman presents with 5-day history of low-grade fever, malaise, and cough and recent chest pain. He has a triphasic pericardial rub and concave-up ST elevation in most of the ECG leads. ESR is 96 mm at the end of first hour, and CBC is normal. Serum troponin I level is 15 times the normal. What is the most likely cause of his pericarditis?

{~ Idiopathic

=Viral

~Bacterial

~ Tubercular}

::Q16:: A 25-year-old female patient comes to your OPD. She has a history of chronic kidney disease (CKD) stage 2. She denies any cardiac symptoms. Her vitals are heart rate 76 bpm, BP 170/98 mmHg, and respiratory rate 14/min. Her physical exam is otherwise normal. She is on furosemide 40 mg daily. What would your add-on drug of choice in this patient be?

{~Amlodipine

~Clonidine

= Lisinopril

~ Hydralazine}

::Q17:: A 49-year-old male merchant from Gondar town comes to your office for an evaluation as part of his medical check-up. He reports feeling well. He denies previous history of diabetes or coronary artery disease. Physical examination is unremarkable except for his BP, which was measured at 165/98 mmHg. His BP rechecked after 5 min was 160/100 mmHg. You ordered echocardiography and result turned with mild LV hypertrophy and grade I diastolic dysfunction. What do you recommend for this patient?

{~ Reassurance

=Initiate drug therapy

~ Come back for a follow up in 3 months

~come back for a follow up after 6 months}

::Q18:: A 50-year-old female is being evaluated at follow up clinic. She reports a history of diabetes mellitus and is taking metformin 500 mg BID. She reports no other history. She denies any cardiac symptoms. Her examination is unremarkable except for her high BP, which was measured on two occasions 5 min apart and was 170/92 mmHg. What is the initial drug of choice?

{~Prazosin

~Clonidine

~ Atenolol

=Lisinopril}

::Q19:: Which one of the following factors is least likely to affect the cerebral blood flow to the CNS at precapillary level?

{~Systemic BP

~Intracranial Pressure

=Arterial Partial pressure of oxygen

~Arterial Partial pressure of carbon dioxide}

::Q20:: Which one of the following is not an adaptive mechanism to improve cerebral perfusion against an increased intracranial pressure?

{~Increased systemic blood flow to the brain

~Shift of CSF from brain to spinal canal

~Increased venous flow out of intracranial compartment

=Increased CSF production in the brain}

::Q21:: Which one of the following is not true about the normal composition of CSF?

{~Up to five leucocytes

~Protein level of 40 mg/dl

~Glucose level of 20 mg/dl

=CSF pressure of 20 cm of H₂O}

::Q22:: The primary pathophysiologic mechanism of a typical variant of GBS is?

{=Demyelinating injury of large diameter peripheral nerves

~Axonal injury of large diameter peripheral nerves

~Demyelinating injury of small diameter peripheral nerves

~Axonal injury of small diameter peripheral nerves}

::Q23:: Which one of the following pathogenic abnormalities is not observed at the NMJs of a myasthenia gravis patient?

{~Blunted surface of the post-synaptic membrane

~Decreased number of Ach receptors in post-synaptic membrane

~Increased clearance of Ach from the synaptic cleft

=Decreased number of Ach containing vesicles in the pre-synaptic membrane}

::Q24:: A 75 years old male patient admitted with a diagnosis of acute transverse myelopathy. Thoracic MRI showed diffuse swelling of the cord with heterogenous enhancement. CSF examination produced few lymphocytes (20 cells/micro liter), slightly elevated protein (80 mg/dl) and normal glucose level (45 mg/dl). The most likely cause of this disorder is?

{~Idiopathic transverse myelitis

=Viral myelitis

~Bacterial myelitis

~Multiple sclerosis}

::Q25:: Based on the clinical senario in question number 6, what will be the best diagnostic technique will you request to confirm the diagnosis?

{~CSF oligoclonal bands

~CSF Culture

=CSF PCR

`CSF serology}

::Q26:: Which one of the following conditions is not a strong risk factor for small vessel stroke?

{~Hypertension

~Diabetes mellitus

=Chronic kidney disease

~Smoking}

::Q27:: Twenty five years old female patient came with progressive symmetric weakness of all extremities of three month duration. She reported difficulty of combing her hair, walking upstairs and standing from the toilet sink. Examination showed predominant weakness of arm flexion and abduction as well as hip flexion and knee extension bilaterally. Sensory, reflex and cranial nerve examinations were normal. The most likely cause of her problem is?

{=Myopathy

~Neuro-muscular junction disorder

~Motor neuron disease

~Peripheral neuropathy}

::Q28:: The gold standard diagnostic modality for unruptured intracranial aneurysm is?

{~CT angiography

{~MRI angiography

=Digital Subtracted Angiography

~Trans-cranial doppler ultrasound}

::Q29:: Which one of the following clinical evidences is not included in the diagnostic criteria of migraine without aura?

{~Headache attacks of five or more

~Throbbing quality of headache

~Presence of nausea and vomiting

=Retro-orbital location of the headache}

::Q30:: Presence of automatism strongly suggests?

{~Generalized onset seizure

=Focal onset seizure

~Absence seizure

~Atypical absence seizure}

::Q31:: During the medical management of acute ischemic stroke which one of the following strategies is an appropriate measure to salvage the ischemic penumbra?

~Induction of short-lived hypothermia

=Maintenance of adequate hydration

~Maintaining the BP below 140 mmHg

~Keeping RPG below 100 mg/dl}

::Q32:: An epileptic patient with a well-controlled seizure type came to the neurologic clinic with a complaint of recent two episodes of similar seizures episodes in the last week. Which one of the following problems should an intern consider the least?

{~Presence of seizure precipitants

~Adequacy of the drug dosage

=Wrong classification of seizure type

~Acute symptomatic seizure}

::Q33:: Which one of the following clinical tools is not useful in the follow up of patients to consider anticoagulation treatment for an atrial fibrillation patient with cardio-embolic stroke?

~NIHSS Score

~HAS-BLED Score

=CHAD-VASC Score

~ASPECTS Score }

::Q34:: What is the most common and characteristic symptom or sign of liver disease?

{~Jaundice

=Fatigue

~Right upper quadrant pain

~Abdominal distension}

::Q35:: All of the following are indications for urgent endoscopy for acute upper GI bleeding except?

{~Large volume initial bleeding

~Orthostatic hypotension

~Bloody nasogastric aspirate that does not clear with large volume lavage

=Severe pain}

::Q36:: A 30 year old male patient from Gondar town presented with fatigue and malaise, on routine laboratory testing he is found to have an AST of 164 IU/L, ALT of 183 IU/L, an ALP of 120 IU/L, a total bilirubin of 1.2 mg/dl, and an INR of 0.9.

Which of the following best describes the pattern of liver test?

{~ Cholestatic

~Mixed

=Hepatocellular

~Cirrhotic}

::Q37:: A 42 year old female patient presented with a complaint of intermittent abdominal pain, fatigue and malaise and she has history of multiple sexual partners but she denies history of alcohol use and has never used injection drugs. On laboratory AST 72 IU/L, ALT 65 IU/L, Total bilirubin 1.0 mg/dl, INR 0.7, and on hepatitis serology result; HBSAg: Positive; HBSAb: Negative; HepB Core-IgM antibody: Negative; HB core IgG antibody: Positive; HBeAg: Positive; HBeAb: Negative.

Which of the following best describes her hepatitis B status?

{~Chronic hepatitis B, High infectivity

~Acute hepatitis B, Low infectivity

~Chronic hepatitis, low infectivity

~Acute hepatitis, High infectivity}

::Q38:: A 50 year old male known cirrhotic patient secondary to chronic hepatitis C infection currently presented with altered mental status. All of the following can precipitate hepatic encephalopathy in this type of patient except?

{~Hypokalemia

~Medication non-adherence

=Hyperkalemia

~Dehydration}

::Q39:: A 60 year old female patient presented to emergency department with the complaint of dyspepsia, which one of the following symptom or sign is alarming features of dyspepsia?

{~ Epigastric pain

= Unexplained iron deficiency anemia

~Postprandial fullness

~Unexplained diarrhea}

::Q40:: A 45 years old female patient presented with a complaint of burning epigastric pain and post prandial bloating sensation of 2 months duration. Otherwise she had no difficulty of swallowing or weight loss. She has no other complaint. The physical examination was unremarkable. What is the next preferred form of investigation for the possible diagnosis?

{~Upper GI endoscopy

=Stool H. pylori antigen test

~Abdominal ultrasound

~Barium swallow}

::Q41:: Which one of the following factors is not found to be protective of ulcerative colitis?

{~Smoking

~Prior appendectomy

=Oral contraceptives

~Coffee consumption}

::Q42:: A 45 years old male patient came complaining to have experienced difficulty of swallowing while he was eating a burger with his friends at a restaurant. He stated that he had prior history of few similar episodes in the past but otherwise had been healthy. Physical examination and basic investigations are normal. What is the most likely diagnosis?

{`Esophageal cancer

=Schatzki ring

~Achalasia

~Diffuse esophageal spasm}

::Q43:: A 38 years old male patient was told to have H. pylori after stool antigen test was done and started on triple therapy. When do you confirm the eradication of the bacteria?

{~Immediately after completion of the antibiotics

~At least after 2 weeks of completion of the antibiotics

=At least after 1-2 weeks of discontinuation of the PPI

~No need of confirming eradication}

::Q44:: The most common complication associated with PUD

{=GI bleeding

~Perforation

~GOO

~Penetration}

::Q45:: A 44-year-old man comes to the emergency department with abdominal pain and nausea for the past 24 hours. The pain started a few hours after dinner, and he describes it as constant pain in the upper part of his abdomen that radiates to his back and flanks. The patient also reports nausea,

intermittent vomiting, and abdominal distention. He reports drinking an average of 6 alcoholic drinks per week. His temperature is 37.7°C, pulse is 106/min, respirations are 20/min, SPO2 is 96%, and blood pressure is 149/94 mmHg. He appears distressed and anxious. Abdominal examination shows tenderness and rigidity over the epigastric and periumbilical regions. Bowel sounds are diminished. No bruising of the umbilicus or flanks are seen. Which of the following laboratory results is the most specific in diagnosing this patient's condition?

{~Serum amylase > 3 times the upper limit of normal

=Serum lipase > 3 times the upper limit of normal

C~ Serum bilirubin > 4.0 mg/dL

D~ Elevated blood alcohol level}

::Q46:: Patients with which disease are called pink puffers

{~Bronchiectasis

~Asthma

~Chronic bronchitis

=Emphysema}

::Q47:: Which of the following COPD flare is an indication to start antibiotics?

{=Dyspnea and change in quantity or character of sputum

~Tachypnea and change in quantity or character of sputum

~Orthopnea and low oxygen saturation

~Dyspnea and low oxygen saturation}

::Q48:: Which of the following would increase in obstructive lung disease but not in restrictive lung disease?

{~Ratio of FEV1/FVC

=Residual volume

~FVC

~FEV1}

::Q49:: What is the treatment for acute exacerbation of bronchiectasis?

{=Oxygen, hydration, antibiotics, chest physiotherapy

~Corticosteroids, hydration, antibiotics, chest physiotherapy

~Oxygen, hydration, antibiotics and inhaled bronchodilator

~Oxygen, antibiotics, chest physiotherapy, inhaled bronchodilator}

::Q50:: A 62-year-old woman is hospitalized following an acute pulmonary embolism. All of the following would typically indicate a massive pulmonary embolism EXCEPT

{=Initial presentation with hemoptysis

~Initial presentation with syncope

~Presence of RV enlargement on CT scan of the chest

~Presence of RV hypokinesis on echocardiogram}

::Q51:: A person with Legionella pneumophila can be best treated with:

{~Vancomycin

~linezolid

=Azithromycin

~ceftazidime}

::Q52:: All of the following are the most common pathogens that can cause pharyngitis/sore throat, Except?

{~Corynebacterium diphtheriae

~Streptococcus pyogenes

~Adenovirus

=Candida albicans}

::Q53:: A 62-year-old woman is admitted to the hospital with a community-acquired pneumonia with a 4-day history of fever, cough, and right-sided pleuritic chest pain. The admission chest x-ray identifies a right lower and middle lobe infiltrate with an associated effusion. All of the following characteristics of the pleural effusion indicate a complicated effusion that may require tube thoracostomy EXCEPT:

{~Pleural fluid pH <7.20

~Pleural fluid glucose <60 mg/dL

~Positive Gram stain or culture of the pleural fluid

=!Recurrence of fluid following the initial thoracentesis}

::Q54:: A 24-year-old man come regular OPD with a complaint of episodic and recurrent shortness of breath with associated productive cough which commonly worsens during sleep time. His family history is remarkable for her mother has allergic Rhinitis. No fever or contact history with TB patients. On physical examination, the patient is stable with scattered wheeze all over the chest. What will the NEXT step to confirm that he could have bronchial asthma?

{~Chest X ray or HRCT

=Bronchodilator test

~Arterial blood gas analysis

~Eosinophilia count}

::Q55:: A 50-year-old house wife present with worsening of productive cough, intermittent dyspnoea for the past 6 year. She had multiple visits to the nearby health facilities and was receiving antibiotic for the diagnosis of pneumonia which shows some improving. On physical examination, BP= 120/70 PR= 112 RR= 30 SpO2 = 86- 89 % with room air. She had coarse crepitation on bilateral chest and P2 accentuation on cardiac evaluation, otherwise Normal. The patient finally diagnosed as COPD. Which of the following shows little or No effect in the management of this patient at Outpatient?

{~Oxygen

~Albuterol as needed

=Inhaled corticosteroid alone

~Inhale corticosteroid with Long acting beta agonist}

::Q56:: A 30 year old known asthmatic woman came at gestational age of 10 weeks with shortness of breath and cough which occur 3-4 times a week over the last 3 weeks. Her vitals are stable with oxygen saturation of 95% at room air. She has scattered wheezes on bilateral posterior chest. What is the most appropriate management for this patient?

{~Short acting beta-2 Agonists

=Short acting beta-2 Agonists with inhaled Corticosteroids

~Intravenous Corticosteroids

~Adrenaline}

::Q57:: A 50 years old male patient from koladiba presented with low grade fever, weight loss, night sweat and dry cough. From the evaluation disseminated tuberculosis was diagnosed and plan to start anti-TB. What condition is an indicator to start pyridoxine as an add on treatment during initiation of anti- TB treatment?

{~Number of site involved by the dissemination

~ The chest x ray finding

~CNS involvement

=The presence of comorbidities}

::Q58:: A 40-year-old HIV patient, on TDF/ 3TC/DTG since two months, is evaluated for pulmonary tuberculosis after presenting with cough and fever of three weeks duration. Having a positive sputum GeneXpert, he is directly linked to a TB clinic for anti-TB treatment initiation. His baseline CD4 count was 200/microliter. Which of the following is the most appropriate strategy in the management of this patient?

{~Replace DTG with EFV

~Temporarily hold HAART, send HIV viral load test
=Continue the same ARVs, double the dose of DTG
~Refer to ART clinic for second line ARV switch}

::Q59:: A medical student presents with fever, headache, arthralgia and sore throat of 5 days duration. Physical exam reveals conjunctivitis, erythematous and swollen pharynx, and mild splenomegaly. CBC and peripheral blood exam are notable for a WBC of 2200 (neutrophil: 30%, lymphocytes: 68%) and atypical lymphocytes. Of the following choices, which one is the most likely diagnosis?

{~Bacterial pharyngitis
~COVID-19
=Infectious mononucleosis
~Croup}

::Q60:: While working in a remote district hospital, a young lady on her second trimester of pregnancy, comes with fever and headache of four days duration. Vital signs are: T: 38 C, BP: 90/60 mmHg, PR: 110 bpm, RR: 28/min. Physical exam shows pale conjunctiva, icteric sclera, splenomegaly and basal rales. The only lab test available at the facility RDT, is positive for malarial antigens. What is the most appropriate drug treatment?

{=IV artesunate x 5 days + Primaquine single dose
~IV quinine x 5 days + Doxycycline x 5 days
~Coartem x 3 days + Artemeter IM stat
~Chloroquine x 3 days + Clindamycin x 5 days}

::Q61:: A 30-year-old farmer complains watery diarrhea and abdominal distension of five days duration. Stool microscopy shows trophozoites of Giardia Lamblia. What is the best treatment?

{~Mebendazole 200 mg stat
=Tinidazole 2g stat
~Albendazole 400 mg x 3 days
~Metronidazole 500 mg TID x 7 days}

::Q62:: A young boy comes with fever, headache and bizarre behaviour of six days duration. Physical exam is unremarkable. CSF analysis shows WBC of 300 with 75% lymphocytes. Gram stain and culture reveal no organism.

What is the most likely etiology?

- {~Listeria monocytogen
- ~Adenovirus
- =Herpes simplex virus
- ~N. meningitides}

::Q63:: A 47-year-old male with quadriplegia secondary to a motor vehicle accident as a young adult presents with fever, fatigue, and foul-smelling urine. He has a chronic indwelling Foley catheter due to urinary retention, and he reports multiple hospitalizations for catheter-associated urinary tract infections. On arrival to the emergency department, he is noted to be confused with a temperature of 38.3°C, a RR of 23 BPM, a HR of 116 BPM, and a BP of 75/43 mm Hg. Urinalysis finds 3+ leukocyte esterase and positive nitrite with microscopy revealing 100–200 WBC per HPF with 3+ bacteria. Blood and urine cultures are obtained and laboratory findings are pending.

In addition to administering broad-spectrum antibiotics, the next best step is:

- {~Initiation of dopamine as a vasopressor
- ~Administration of intravenous hydrocortisone
- =Administration of 30 mL/kg of crystalloid fluids
- ~Initiation of norepinephrine as a vasopressor}

::Q64:: A 53-year-old woman with well-controlled non–insulin-dependent diabetes and obesity develops an area of swelling and pain on the right thigh. Over about 3 days, the area becomes fluctuant and she develops a large area of erythema around it. She presents to the emergency room, where she is found to have a low-grade fever but is otherwise stable. The collection, which measures 2.5 cm in diameter, is lanced, producing a small amount of purulent material, which is drained and swabbed for culture. The culture of the wound later grows *Staphylococcus aureus*. Which of the following antibiotics along with excellent wound care, would be the best choices for outpatient treatment of this infection?

- {~Levofloxacin
- ~Clarithromycin
- =Trimethoprim-sulfamethoxazole

~Penicillin}

::Q65:: A 25-year-old woman known chronic rheumatic valvular heart disease patient presents to the emergency department with the complaint of high grade fever with muscle aches of 5 days' duration with worsening of heart failure symptoms. The patient had recent tooth extraction at local traditional healer. On examination she is tachycardic (130BPM), diaphoretic, febrile (38.8°C), and blood pressure of 100/60 mm Hg. She appears to be acutely sick. Cardiac examination reveals a new systolic murmur, and raised jugular venous pressure. Given her ill appearance, the admitting physician decides to start empiric antibiotics immediately.

Which one of the following investigation have to be sent for laboratory before the antibiotic is given?

{=At least two set of Blood culture from separate venipuncture sites.

~Single set of blood culture from single venipuncture site

~Complete blood count with CRP

~Transthoracic Echocardiography}

::Q66:: A 28 years old woman presented to the emergency with sever repeated vomiting and abdominal cramp around 2 hours after eating food from a wedding buffet which had a range of hot and cold foods, also meat and dairy products. On examination her BP is 90/60 mmHg. And PR was 102BPM and regular. And she had vomiting during the examination? Which one of the following is the most likely cause of her food poisoning?

{~Salmonella enteritis

=Staphylococcus aureus toxin

~Vibrio cholera

~Shigella sonnei }

::Q67:: A 20 years' young man with epilepsy presented with a 3-day history of cough with foul smelling sputum and increasing shortness of breath. He had an episode of generalized tonic clonic seizure 01 weeks back. On examination he was febrile (39°C), tachycardia with HR (100bpm), tachypnea with RR of

28 BPM and oxygen saturation of 80%, he has right sided crepitation and his chest X-ray shows a right upper lobe infiltration. Which one of the following is the most likely cause of pneumonia for this patient?

{~Chemical pneumonitis

~Aspiration mycoplasma pneumonia

~Aspiration pneumonia due to gram negative aerobic bacteria's

=Aspiration pneumonia due to anaerobic bacteria's}

::Q68:: A bank manager came to the outpatient department for routine evaluation. All lab tests are normal but her TSH is elevated. She denied any history of apparent hypothyroidism symptoms. Which one of the following is clear indication for the treatment?

{~TSH >5 mIU/L

~Middle aged man

=Pregnancy

~Risk of heart disease}

::Q69:: A 35-year-old female patient present with excessive weight gain in the trunk and face with easily bruising and a fatty lump between the shoulders. Lab studies showed an elevated plasma corticotropin (ACTH) >20 pg/mL (4.4 pmol/L) and Cushing syndrome is considered. Which one of the causes of Cushing syndrome is ACTH dependent?

{~Adrenal adenomas

=Pituitary - dependent disease

~Glucocorticoid administration

~Alcohol - induced Pseudo - Cushing's syndrome}

::Q70:: A 40-year-old female patient has presented with a polyuria of 4.2 liters per 24 hours and nocturia of thrice per night. The serum sodium is high normal range; urine specific gravity is 1.001 and urine osmolality of less than 300 mosm/L. Water deprivation test shows urine osmolality of <300 mosm/L both before and after desmopressin. Which one of the following is the diagnosis?

{~Dypsogenic diabetes insipidus

~Pituitary diabetes insipidus

=Nephrogenic diabetes insipidus

~Hypothalamic diabetes insipidus}

::Q71:: A 25 years old patient presented with diffuse goiter, Lid retraction and tremor. TSH is low and raised thyroid stimulating antibodies. Which one of the following is the likely diagnosis?

{=Graves' disease

~Hashimoto's thyroiditis

~Toxic adenoma

~Toxic multinodular goiter}

::Q72:: An 18-year-old male patient presented with right upper quadrant pain and an abnormal transaminase level. Later he developed peripheral stigma of chronic liver disease and dysarthria and gait abnormalities. Which one of the following is true about this patient?

{~Serum ceruloplasmin level will be high

~24-hr urinary copper excretion will be decreased

~Ocular slit-lamp examination confirms the diagnosis

=Untreated, the disease is universally fatal}

::Q73:: A 39 years old female patient presented with infertility, galactorrhea and amenorrhea. Which one of the following is true about this patient?

{~Prolactinoma is highly likely with a prolactin level $<20 \mu\text{g/L}$

~A normalization of prolactin level always shows reduction in tumor size

~Asymptomatic microadenomas should be treated as most progress to macroadenoma

=Tumor size generally correlates directly with PRL concentrations}

::Q74:: A 27-year-old man comes to office due to recurrent episodes of muscle weakness. he has no other significant medical history. The patient's weight has been stable for the past few years, and his current BMI is 23kg/m². His BP is 190/110 supine and 195/110 mmHg standing. His heart rate 70/min supine and 72/min standing. The rest of the physical examination is unremarkable. Laboratory shows very low renin activity. Overactivity of which of the following structures is most likely responsible for this patient's symptoms?

- {~Chromaffin cells of the adrenals
- ~Juxtaglomerular cells of the kidney
- ~Zona fasciculata of the adrenals
- =Zona glomerulosa of the adrenals}

::Q75:: A 24yrs old man diagnosed with diabetes mellitus 2yrs ago is brought to the emergency department after a temporary loss of consciousness. He was getting ready to eat and administered a preprandial insulin injection , but he was distracted by a phone call from work and ended up skipping the meal. his girlfriend found him passed out in his bedroom and administered glucagon immediately and the patient recovered consciousness within 15 minutes. Metabolic changes in which of the following organs are most likely responsible for this patient's recovery?

- {=Liver
- ~Skeletal muscle
- ~Small intestine
- ~Pancreas}

::Q76:: A 65-year old man comes to the physician because of a non-healing ulcer on his right foot. His past medical history is significant for recurrent high blood glucose readings for the last several years , because he has failed to comply with appropriate treatment. His BMI is 37kg/m² . Physical examination is significant for bilateral symmetric decrease in vibration sensation over the feet and ankles. Which of the following is most likely to be associated with this patient's condition?

- {~Pancreatic islet infiltration with leukocytes
- ~Circulating anti-islet antibodies
- =Pancreatic islet amyloid deposition
- ~Strong linkage with HLA class II gene makeup}

::Q77:: A 58 year-old male known type 2 DM patient since the past 5 years on metformin and atorvastatin . On his medical history review, he was diagnosed with IHD and on enalapril and ASA since the past 2years.on His home blood measurement , he has uncontrolled FBS and HbA1c is 9%. BP is 140/90 and no other pertinent physical findings. OFT is normal. Which one of the following is the best treatment option to control his blood sugar level?

- {`Insulin
- ~Sulfonylureas
- =SGLT-2 inhibitors
- ~DPP-4 inhibitors}

::Q78:: Which of the following is the MAIN CAUSE of anemia in patients with chronic kidney diseases(CKD)?

- {~Folic acid deficiency
- =Erythropoietin deficiency
- ~Iron deficiency
- ~Blood loss}

::Q79:: A-50-year-old male presented with fever and cough of one week duration. On examination he was febrile, tachycardic and blood pressure was normal. Investigations revealed serum creatinine of 1.7mg/dl. Which of the following urine microscopy findings strongly supports the diagnosis of acute tubular necrosis (ATN) in this patient?

- {~Red blood cell casts
- ~White blood cell casts
- =Muddy brown granular casts
- ~Hyaline casts}

::Q80:: A65-year-old female known Type 2 diabetic and CKD patient with eGFR of 45ml/min/1.72m² came for routine follow up. Current medications are: Metformin 1gm bid, Amlodipine 5mg/day, Lisinopril 10mg po bid and Atorvastatin 40mg po/day. Her current HA1c is 8.5% and 24 hr urine protein is 3.2gm/day. Which of the following is the MOST appropriate management at this stage?

{~Add Enalapril

~Add Insulin

~Add Glimepiride

=Add Dapagliflozin }

::Q81:: A 20-year-old male presented with road traffic accident of one hour duration. On examination he has bleeding from multiple crush injuries and muscle damage on lower extremities. Blood pressure is 100/80mmHG. The following day he has fatigue and up on catheterization, he passes 50ml of red to brown color urine. Serum creatinine is 1.8mg/dl and serum potassium is 6.6meq/L. What is the main stay of therapy for this patient at this stage?

{=Hydration

~Hemodialysis

~IV furosemide

~IV bicarbonate}

::Q82:: A 35-year-old female presented with generalized body swelling of 1 week duration that started from her face. Blood pressure is 140/80mmHg and has pleural effusion, ascites and peripheral edema. Urinalysis showed Albumin 3+, no active sediments, 24 hr protein of 3.6gm/day. Serum creatinine is 1.1mg/dl, Albumin 2.2mg/dl and total cholesterol was 288mg/dl. What is the syndromic diagnosis of this patient?

{~Rapidly progressing glomerulonephritis

~Acute nephritic syndrome

=Nephrotic syndrome

~Acute kidney injury}

::Q83:: A 50-year-old known patient with decompensated chronic liver disease (CLD) on follow up was noted to have serum sodium of 120meq/L. He has ascites and peripheral edema. Which of the following is the MOST appropriate measure in the treatment of this patient's electrolyte disturbance?

{=Restriction of free water intake

~Oral table salt

~IV 3% sodium chloride

~Vasopressin receptor antagonists}

::Q84:: Chronically ill patient presented with easily fatigue and lassitude. He had conjunctival and palmar pallor. Laboratory profile revealed microcytic red cells with reduced serum Iron, TIBC, and % transferrin saturation. Likely diagnosis would be

{~Iron deficiency anemia

=Anemia of inflammation

~Thalassemia

~Sideroblastic anemia}

::Q85:: Non-vegetarian elderly patient diagnosed to have megaloblastic anemia with low serum cobalamin level, but normal cobalamin absorption. Possible explanation includes

{~Poor dietary intake

~Stagnant loop syndrome

~Chronic pancreatitis

=Pernicious anemia}

::Q86:: The most common hematologic complication in HIV infected patients

{=Anemia

~Neutropenia

~Monocytopenia

~Thrombocytopenia}

::Q87:: A 50-year old male patient presented to outpatient clinic with aquagenic pruritis of 2 months duration. On physical examination, he had plethoric face with symptomatic splenomegaly. Laboratory values revealed increased trilineage blood cells. The treatment of choice is

{~Chemotherapy

=Phlebotomy

~JAK-2 inhibitors

~Bone marrow transplant}

::Q88:: Characteristic features of non-Hodgkin lymphoma includes

{~Centripetal distribution

=Non-contagious nodal spread

~B symptoms are common

~Bone marrow involvement affects prognosis}

::Q89:: The documented risk factor for chronic myeloid leukemia

{~Industrial chemical exposure

~Genetic (familial) predisposition

~Chemotherapeutic agents

=Ionizing radiation}

::Q90:: The most frequent reaction associated with the transfusion of cellular blood component is

{~Delayed hemolytic and serologic transfusion reaction

=Febrile non-hemolytic transfusion reaction

~Allergic reaction

~Acute hemolytic transfusion reaction}

::Q91:: Which of the following is true about non inflammatory arthritis?

{~Synovial fluid viscosity is decreased on the affected joints.

=Intermittent and short duration of arthritis and stiffness is characteristic

~Acute-phase reactant such as the ESR or CRP are usually elevated

~Joint involvement is always mono or pauciarticular}

::Q92:: A 25-year-old female patient presented with pain and swelling over bilateral hand joints over the past 8 weeks associated with pronounced morning stiffness. The physical exam was remarkable for tender and swollen bilateral metacarpophalangeal and proximal interphalangeal Joints .

Which investigation is more reliable to make a diagnosis of this patient?

{~Bilateral hand joint X-ray

~ESR

~Rheumatoid factor

=Anti-CCP antibody}

::Q93:: Based on the case presented on question No. 3, which medication is the cornerstone in the management of the above patient?

{~NSAIDS

~Glucocorticoids

=Methotrexate

~Azathioprine}

::Q94:: A 30-year-old female university student presented with easy fatigability, knee joint pain and photosensitivity for a while. Objective evaluation revealed a disc shaped rash over her posterior chest but no joint swelling or deformity. Investigations done showed +2 proteinuria from urine, but ANA was negative. The patient didn't show any improvement either despite taking NSAIDS for quite some time. What is the most appropriate next step approach for this patient?

{~Perform renal biopsy

~Determine Double stranded DNA

=Repeat Anti-nuclear antibody (ANA)

~Initiate treatment for SLE and follow for response}

::Q95:: Which of the following complication is the most likely cause of mortality in SLE patients?

{=Renal

~Hematology

~Respiratory

~Musculoskeletal }

::Q96:: Which of the following types of arthritis is least likely to involve the distal interphalangeal joints?

{~Osteoarthritis

~Reactive arthritis

~Psoriatic arthritis

=Rheumatoid arthritis }

::Q97:: A teenager who presented with mildy itchy skin lesions was examined to have oily skin, comedones, papules and nodulocystic lesions on the face and upper trunk. What is the first line treatment for this patient?

{~Topical retinoids

~Topical antibiotics

~Oral doxycycline

=Oral Isotretinoin}

::Q98:: A middle aged man presented with skin lesions and numbness of the extremities. On evaluation you find a single well demarcated anaesthetic annular plaque on his trunk along with a unilateral ulnar nerve thickening & enlargement. Skin slit microscopy was negative for AFB. What is the best management?

{~Rifampicin, clofazimine and dapsone for 06 months

=Rifampicin and dapsone for 06 months

~Rifampicin, clofazimine and dapsone for 12 months

~Rifampicin and dapsone for 12 months}

::Q99:: A 28 year old male with a positive family history of itching presented with a generalized itchy skin rash. On P/E the whole skin is involved with whitish crusts and excoriated papules while the palmoplantar skin is hyperkeratotic. Further evaluation with microscopy showed adult scabies mites and eggs. What is the most appropriate treatment for this case?

{=Ivermectin

~Prednisolone

~1% Lindane lotion

~5% permethrin cream}

::Q100:: A 24 year lady presented with an asymptomatic generalized skin rash and oro-genital lesions. On evaluation you find a maculopapular rash with palmoplantar involvement, whitish oral mucosal patches and condylomata lata. What is the most likely diagnosis?

{~Primary syphilis

=Secondary syphilis

~Tertiary syphilis

~Genital warts}

::Q101:: A twenty five years old female patient came to the emergency with abdominal pain which started at the center of abdomen and later shifted to the right lower quadrant of 3 days duration. She also has low grade fever, anorexia and three episodes of bilious vomiting. On physical exam, she is acutely sick with a pulse rate of 112 and a temperature of 38⁰C. What is the most likely diagnosis?

{~Acute cholecystitis

~Acute pancreatitis

=Acute appendicitis

~Mesenteric adenitis}

::Q102:: A sixty five years old male farmer came to the clinic with a sudden onset abdominal pain which started at the epigastric area of one day duration. The pain is severe and has involved the lower

abdomen for the past six hours. He also has one episode of nonprojectile vomiting of ingested matter and low grade fever. Physical exam revealed acutely sick old man with blood pressure of 80 by 50 millimeter of mercury, pulse rate of 130 beats per minute and temperature of 39 degree Celsius. The abdomen is tender all over with rigidity. What is the most appropriate next step in the management of this patient?

- {~Sending for plain abdominal x ray
- ~Sending blood for laboratory
- =Resuscitation with crystalloids
- ~Doing exploratory laparotomy}

::Q103:: A 55-year-old female patient came to the clinic with a complaint of anterior neck swelling of six years which initially was small but later grows rapidly over the past year to bigger size. The swelling is painless with no other symptoms. Objective evaluation findings include normal vital signs and a two by one centimeter anterior neck mass on the right side with hard consistency and round border. What is the most important diagnostic investigation?

- {~Thyroid function test
- =Fine needle aspiration biopsy
- ~Radioiodine scanning
- ~Neck ultrasound}

::Q104:: A 30-year-old nursing mother came to the clinic with left breast pain and swelling of five days duration with low grade fever and pain. On physical exam she is acutely sick with a pulse rate of 110 and a temperature of 39 degree Celsius. Complete blood count showed a white cell count of eighteen thousand with ninety percent granulocyte. Breast ultrasound showed a six by five centimeter hypoechoic lesion on the upper outer quadrant. What is the most likely etiology for the patient condition?

- {~Escherichia coli
- ~Streptococcus pyogenes
- =Staphylococcus aureus
- ~pseudomonas aeruginosa}

::Q105:: A 45-year-old female patient came to the clinic with a right upper quadrant abdominal pain of seven days duration which initially was intermittent but later became severe and constant for the past two days. She also has low grade fever, anorexia and one episode of vomiting of ingested matter. Physical exam revealed a pulse rate of 110 with a temperature of 38 degree Celsius. She has tender right upper abdomen. The serum bilirubin level is 7 milligram per deciliter with alkaline phosphatase of 1000. What is the most likely diagnosis?

- {~Acute cholecystitis
- ~Acute hepatitis
- =Acute cholangitis
- ~Gall bladder empyema}

::Q106:: A 28Years old man brought to surgical emergency ward having frequent bilious vomiting, crampy abdominal pain and failure to pass feces and flatus of twelve hours duration. On physical exam; He was acutely sick looking, Pulse rate was 116 beats per minute, Blood pressure was 90 mmHg by 55 mmHg and his temperature was 37.9. On abdominal examination there was distended abdomen with visible bowel loops but no tenderness. What could be the initial management for this patient?

- {~Laparotomy before ischemia starts
- =Fluid resuscitation and decompression
- ~Investigate with plain abdominal x ray
- ~Put him on IV antibiotics and follow}

::Q107:: A 52 Year old woman brought to surgical ED presented with two episodes of vomiting, nausea and crampy abdominal pain of eighteen hours duration. On physical exam He was acutely sick looking, has muddy sclera, PR was 106 beats per minute, BP was 100 mmHg by 75 mmHg and her temperature was 38.7°C. On abdomen, there was tenderness over RUQ. What could be the initial management for this patient?

- {~Investigate with CBC and ultrasound as an out patient
- ~Resuscitate her and surgery with suspicion of cholecystitis
- =Fluids, IV antibiotics, analgesia and investigate at emergency
- ~Emergency abdominal CT and PO antibiotics}

::Q108:: A 42-Year-old man brought to surgical ED presented with stab injury to his left lower quadrant after 21 hours duration and the trauma team operated him with the diagnoses of penetrating abdominal injury with PR was 112 beats per minutes, Temperature 38.2 and tender abdomen. Intra operative finding was sigmoid injury with fecal contamination of the peritoneum. After 48 hours of end colostomy and stable vital signs, he becomes febrile, tachycardia and nonfunctional stoma. What kind of post-operative complication he could have?

- {~Immediate post-operative ileus
- =intra-abdominal collection
- ~Lower urinary tract infection
- ~Surgical site wound infection}

::Q109:: A 12 Year old boy brought to surgical emergency department having right lower quadrant clean surgical dressing after he operated for acute gangrenous appendicitis of 36 hours duration. His main complaint was severe pain around the site. On physical exam; he is acutely sick looking, PR 106 beats per minute, BP 110 mmHg by 65 mmHg and his temperature is 38.2 and erythematous with tender surgical wound. Interrupted stitch was there. What could be the initial management for this case?

- {~IV antibiotics and investigate with CBC
- ~PO antibiotics and abdominal ultrasound
- ~IV antibiotics, analgesia and wound care
- =Open the stitch and see the wound}

::Q110:: A 60-year-old man operated with Onlay mesh hernia repair for the diagnoses of right side direct inguinal hernia. He was a daily laborer. Mode of anesthesia was spinal. But after six hours of the operation, he started to complain severe headache and dizziness. PACU monitoring machine showed BP was 110 by 65 mmHg and others are in normal range. What could be the reason for the headache and possible supportive treatment?

- {~Transfusion is better, it could be explained by hemorrhage
- =Adequate fluid is better, it can be explained by spinal anesthesia
- ~Just give PO analgesia and wait until anesthesia is wear off
- ~Early ambulation can be better than other treatment options.}

::Q111:: A 5-year old child who has been vomiting for 2 days presented to the pediatric emergency. She is thin, pale, lethargic and febrile. She has a diffusely tender, rigid abdomen without localizing signs. Next step in the management of this patient is

- {~Order an abdominal/pelvic CT scan with Oral contrast
- ~Order a barium enema to rule out intussusception
- ~Start fluid resuscitation and observe on broad spectrum IV antibiotics
- =Take her immediately to the operating room for exploration}

::Q112:: A 12 year old girl presents to the emergency department following a fall accident in which the left side of her midtorso hit the branch of a tree. She presented with left side lower chest and upper abdominal pain. She also complains of left shoulder pain. The most likely diagnosis is?

- {~Rib fractures
- ~Liver injury
- ~Ruptured diaphragm
- =Splenic injury}

::Q113:: A 30 years old male presented to emergency room after road traffic accident that happened 2 hours prior to presentation. He had brief period of confusion with episodes of vomiting which was followed by state of non-respondent to calls or touching but no abnormal body movement. On neurologic examination he was opening his eyes only to pain and showing abnormal flexion and moaning for pain. There was clear right side extremity movement preference than the left. Please answer the following questions based on this case. What is the GCS of this patient?

- {~5
- =7
- ~6
- ~9}

::Q114:: His CT scan revealed hyper dense left fronto-temporal biconvex shaped extra-axial collection of 6cm thick with 1cm midline shift at level of foramen of Monroe. Which of the following is true?

- {~The hematoma is typical of acute subdural hematoma
- ~This hematoma can be managed conservatively without surgical evacuation
- =The commonest cause of this hematoma is rupture of middle meningeal artery

~The recommended type of surgical evacuation for this hematoma is a burr-hole}

::Q115:: A 25-year-old male patient presented with cough fever and right-side chest pain of 02 weeks examination revealed PR of 110 and fever highest measurement of 39.1 deg. Centigrade with decreased air entry at the right lower lung field, CXR showed right side effusion and admitted with diagnosis of CAP with PPE.Which of the following is the next appropriate investigation stage the PPE?

~pleural tap and analysis

=Chest US

~CECT of the chest

~Thoracic MRI}

::Q116:: A 40-year-old female who had been treated for cavitary pulmonary tuberculosis 5 years back, presented with hemoptysis of 500ml per 24 hours of one day duration VS at presentation was PR 110 and BP of 90/60 mmHg, Sapo2 of 79% Which of the following is the most likely source of bleeding in this patient?

{=Bronchial arteries

~Pulmonary arteries

~Intercostal collaterals

~Pulmonary veins}

::Q117:: Which of the following is best strategy in managing pain in a patient who is admitted to the recovery with diagnosis of right-side 7th and 8th rib fracture, pulmonary contusion following RTA ?

{~Single NSAID with maximum dose escalation

~Combination of two or more NSAIDS with dose escalation

~Combination of two or more NSAIDS and Opioids

=Combination of regional block, NSAIDS, and opioids}

::Q118:: A 70-year-old man who is known to have Diabetes, hypertension, atrial fibrillation on medical management started to complain sudden onset of pain in the right calf and foot since of eight hours durations and admitted with diagnosis of ALI.Which of the following is the most likely cause of arterial insufficiency?

{=Cardio-embolic phenomena

- ~Complicated arteriosclerosis
- ~Arterio- embolic phenomena
- ~Diabetic microangiopathy}

::Q119:: A 69-year-old diabetic man presents with a 5-day history of mild dysuria, frequency and and pain around genitalia. On examination, he is found to have fever and tachycardia. A genital examination reveals both the penis and the scrotum to be swollen, red and tender to touch, with erythema also extending into the groin bilaterally, there is palpable crepitus in the perineum. He is taken into the operating theatre for definitive management. The most likely diagnosis in this patient is:

- {=Fournier's gangrene
- ~Epididymo-orchitis
- ~Testicular torsion
- ~Testicular tumor}

::Q120:: All of the following statements are true except one:

- {=Painful hematuria indicates malignant pathology
- ~Pain from the urinary bladder is supra pubic discomfort
- ~Most urinary calculi are radio opaque
- ~IVU can be dangerous}

::Q121:: In a patient, Rinne test positive in both ears, Weber's lateralizes to the right. This implies:

- {Right sensorineural deafness
- =Left sensorineural deafness
- ~Right conductive deafness
- ~Left conductive deafness}

::Q122:: An infant has biphasic stridor, barking cough and difficulty in breathing since 3–4 days. He has high grade fever and leukocyte count is increased. Which of the following is wrong statement regarding the clinical condition of the child?

- {~It is more common in boys than in girls
- ~Subglottic area is the common site of involvement
- =Antibiotics are mainstay of treatment

~Narrowing of subglottic space with ballooning of hypopharynx is seen}

::Q123:: Which of the following measures should never be taken before referring a patient with traumatic globe rupture

{~Shield the eye

~Give tetanus prophylaxis

=Apply antibiotic eye ointment or eye drop to prevent infection

~X ray orbit if intra ocular foreign body is suspected}

::Q124:: In SAFE strategy in the management of trachoma, all are true except

{=Antibiotics are used for the treatment of all grades of trachoma

~Surgical correction of the eyelids is needed in patients with trachoma trichiasis

~Personal hygiene and facial cleanliness are the core practices in the prevention of trachoma

~Improving environmental condition that help in controlling of flies' breeding is an essential component of the strategy }

::Q125:: Which one of the following is the gold standard treatment option for a 45 year old man who sustained femur shaft fracture?

{~Pop cast

~Plate and screw

~Traction

=Intramedullary nail }

::Q126:: A 12-year-old boy presents to the pediatric emergency department with pain in his right lower extremity. He was recently playing in soccer, but he denies any trauma to the leg. Physical examination is notable for mild swelling in the right mid-tibia and tenderness to palpation. A radiograph of the leg demonstrates periosteal elevation and onion skinning" appearance. What will be the treatment modality?

{~Radical surgery-Radiotherapy-chemotherapy

~Chemotherapy-radical surgery-Radiotherapy

=Chemotherapy-Radical surgery-Chemotherapy

~Chemotherapy-Radiotherapy-Radical surgery}

::Q127:: A patient came to an emergency department and up on evaluation you get, he has isolated lateral leg compartment syndrome. If you left this disease untreated, which of the following deficit would likely occur later as?

{~Inability to dorsiflex ankle

~Loss of sensation over first web space

=Loss of sensation over the dorsum of the foot

~Inability to plantar flex the ankle}

::Q128:: One of your friend sustained blunt trauma to his arm and immediately you noticed wrist drop. Which nerve most likely got injured?

{~Median nerve

~Ulnar nerve

=Radial nerve

~Musculocutaneous nerve}

::Q129:: An 10-day-old infant is admitted to your hospital for septic arthritis of the hip. He went home on day two after an uneventful vaginal birth. Which of the following will most likely be the causative organism?

{=Group B streptococcus

~S. Aerus

~E. Coli

~H. Influenza}

::Q130:: A 14 year old girl presented to Orthopedic emergency with the complaint of knee pain and limping of 4 days duration. On physical exam you get her pulse rate:118, temprature:38.7°C and grossly swollen knee joint. Which of the following is least important diagnostic morality to reach your diagnosis?

{~Knee ultrasound
~CBC
=Knee x-ray
``~Synovial fluid analysis}

::Q131:: If continued bleeding occurs during the third stages of labor with a contracted uterus, the cause is most likely to be :

{~Placental Abruption
~Uterine atony
~Cervical Polyp
=Cervical and perineal laceration}

::Q132:: With regard to fractures in the newborn period, which of the following statement is true?

{~ Fractures to the long bones are a common finding when there is a history of a difficult delivery.
~Skull fractures found immediately after delivery indicates medical negligence has occurred.
=Rib fractures are rarely caused by CPR in full-term infants, and the possibility of non-accidental injury should be considered
~ Fractures to the humerus or femur will require surgical stabilization to prevent future deformity.}

::Q133:: A 29-year-old para I lactating mother came to the postnatal clinic on the 6th post-partum day to have a contraceptive method. She told to the Intern physician that she has lower abdominal pain, foul-smelling discharge, and low-grade intermittent fever. Regarding the choice of contraception for this client, all are false except:

{~IUCD can be provided if a Tubo-ovarian abscess is ruled out
~Implants are category I for this client

~COCs are category III, so it is generally not to use method for her

=Progesterone-only pills are category 2 and can be used}

::Q134:: An 18 years old girl visited a family planning clinic and she was on a combined oral contraceptive, unfortunately, she forgets taking the pills for 4 consecutive days. She was on menses 21 days ago. Which of the following is not true regarding the management of missed pills in this client?

{=Continue the non-hormonal pills for the next 7 days and start the new pack then

~Use a backup method for the next 7 days

~Use emergency contraception if she had sex for the past 5 days

~Take a pill as soon as possible}

::Q135:: Which of the followings is true about labor progress?

{~The time to dilate 1cm in latent phase of labor is equal in patients undergoing induction and in those in spontaneous labor

=The time to dilate 1cm in active phase of labor is equal in patients undergoing induction and in those in spontaneous labor

~The time to dilate 1cm in latent phase of labor is shorter in patients undergoing induction than in those in spontaneous labor

~The time to dilate 1cm in active phase of labor is shorter in patients undergoing induction than in those in spontaneous labor}

::Q136:: One of the following is the most likely reason for abnormal progress of labor in first stage of labor

{~Neuraxial anesthesia

~Fetal malpresentation and malposition

=Hypocontractile uterine activity

~Cephalopelvic disproportion}

::Q137:: An 18 year old mother presented with a complaint of continuous urine leakage of five months duration. She gave birth to dead fetus five months back via cesarean section after she was laboring for about two days at home and one additional day at local health center due to lack of transportation; upon pelvic examination you observed that the vulva is soaked and there are multiple ulcerated areas over the perineal skin and around the thigh. What is the most likely reason for her skin ulceration?

{~Malnutrition

=Dehydration

~Poor hygiene

~Sexually transmitted infection}

::Q138:: A 32 year old para III married woman who was diagnosed to have stage III uterovaginal prolapse was managed with reconstructive pelvic surgery; What postoperative advice you want to provide for this woman?

{=She can resume sexual intercourse after 6 weeks of the surgery

~Since she is young and the risk of surgical failure is very low she can engage in heavy lifting activities

~Future pregnancy should be avoided totally with effective contraceptive

~Pelvic floor exercise does not have additional benefit after reconstructive surgery}

::Q139:: A 60-year-old postmenopausal woman presents for evaluation of genital tract bleeding, which is confirmed as uterine in origin by physical examination. Which diagnostic procedure is a logical first step in her evaluation?

{~Colposcopy

~Diagnostic hysteroscopy

=Transvaginal sonography

~Saline infusion sonography}

::Q140:: A patient presents to the Emergency Department with a one day history of heavy vaginal bleeding. She is tachycardic but not hypotensive, and postural vital signs are stable with change from a supine to sitting position. Physical examination demonstrates bleeding from above the external cervical

os and continued pooling of blood in the vagina. Laboratory studies reveal she is anemic. Which of the following is the most appropriate first-line agent to attempt control of her acute uterine bleeding?

{~ Intravenous tranexamic acid

=Intravenous estrogen

~ Intravenous Gonadotropin-releasing hormone (GnRH) agonist

~ Combination oral contraceptive pill tapering }

::Q141:: What is the underlying etiology of the proteinuria seen with preeclampsia?

{=Increased capillary permeability

~Increased renal artery resistance

~Increased glomerular filtration rate

~Increased systemic vascular resistance}

::Q142:: Which of the following placental hormones may play a role in preterm birth caused by maternal –fetal stress?

{~Estrogen

~Human placental lactogen

~Insulin-like growth hormone

=Corticotropin-releasing hormone}

::Q143:: A 28 year old G2P1 (alive via SVD) presents to your office at 39 weeks of gestation with pushing down pain of 14 hours duration with cervical dilatation of 3 cm and the presentation was breech. You perform a bedside ultrasound and note estimated fetal weight of 3940g complete breech. Otherwise the fetus is in good condition and she had adequate uterine contractions the most appropriate next step in the management of this patient.

{~Follow the progress of labour

=Deliver by cesarean section

~Augmentation of the labour

~Do external cephalic version}

::Q144:: A 26 year old P1 mother who delivered via cesarean section 8 hours back the postnatal ward nurse measures her temperature and it was 38.3°C the nurse calls to tell you. She had cesarean delivery with no complications estimated blood loss was 700ml you review the chart and medical history. The best appropriate next step in management is:

{~CBC,urinalysis,and chest xray

~Paracetamol and observation

~Administer broad spectrum antibiotics

=Examination of the patient}

::Q145:: A 30-year-old laborer, married for 4 years, presented to the gynecology clinic because she had not become pregnant for 2 years. She has regular monthly menstruation with cyclic molumina. Her husband is apparently healthy and has no problems with intercourse. When asked, she said that she had ectopic pregnancy twice and both fallopian tubes were removed. She is afraid that her husband might divorce her. Your area of advice to this woman might be?

{=Invitro fertilization with embryo transfer

~To adopt a child

~To have a tuboplasty performed

~Use gestational carrier}

::Q146:: A 38-year- old woman and her 50-year-old husband come for examination because they have not become pregnant for 6 months after their marriage. The woman stated that she was divorced from her previous partner because of a similar problem a year ago. She has regular menstruation with cyclic menstrual symptoms. She had two induced abortions at her young age. The current partner is diabetic but has no sexual problems. He also has a 5-year-old child by his former wife. What should be done with these couples?

{~Deny them any evaluation rather advice them on timing of sexual intercourse for the next 6 months

~The problem is certainly on the wife's side

=A hysterosalpingography might give a better picture of the woman's problem

~She is too old to seek fertility treatment}

::Q147:: Which factor is responsible for development of New SCJ(Squamo -columnar junction) in pubertal age cervix?

{~Inflammation

~Cervicitis

~HPV infection

=Estrogen}

::Q148:: In both the Pelvic Organ Prolapse Quantification and Baden-Walker Halfway systems, what fixed anatomic reference point is used for measurements and grading?

{~Ischial spine

~Vaginal introitus

=Hymenal ring

~Cervix or apical vaginal vault}

::Q149:: A 21 years old primigravid lady came for ANC complaining decreased fetal movement of 3 days duration. The pregnancy was uneventful and GA from reliable LNMP is 40 weeks. Upon physical examination, the uterus is term sized and FHB was 130 beats/minute. Which of the following antepartum fetal tests are you going to do first?

{~Contraction stress test

=Non stress test

~Biophysical profile

~Maternal fetal kick count}

::Q150:: Normal changes of pregnancy that mimic signs of cardiac disease include the following except

- {~S3 gallop
- ~Leg edema
- ~Tachycardia
- =Diastolic murmur}

::Q151:: A 10 months old male infant from Gondar kebele 16 was admitted to the Emergency room for severe community-acquired pneumonia. CBC reveals moderate iron deficiency. You discharged this infant with a therapeutic dose of iron. Which of the following response do you expect to happen first?

- {~increase in hemoglobin
- =Increase in serum iron
- ~reticulocytosis
- ~Increase in tissue iron }

::Q152:: A 2-year-old girl is brought into the Pediatric clinic by her parents because they have noticed that she has been progressively unsteady on her feet and bumping into walls over the last few weeks. They have also noticed some occasional twitching movements of her arms and legs over that same period that they do not recall seeing it before. On physical exam, you notice that she has difficulty maintaining her gaze on an object, and you palpate a mass in her abdomen. What kind of tumor do you suspect?

- {~Wilms tumor
- =Neuroblastoma
- ~Pheochromocytoma
- ~Metastatic brain tumor}

::Q153:: An 18-month-old female infant is brought to the pediatric OPD for routine evaluation. A screening hemoglobin level was noted to be 4.6 g/dl. The child does appear pale and has a “cranky” disposition, according to the parents. On further history, the child’s parents inform you that her diet consists of approximately 36 ounces (1 Liter) of cow’s milk per day. There is no prior history of anemia

and no history of pica, trauma, or recent blood loss. Her physical examination is notable for mild pallor and 2/6 ejection systolic murmur at the left sterna border. The most likely diagnosis for this child is:

{~Acute lymphoblastic leukemia

=Iron deficiency anemia

~Lead poisoning

~Anemia of chronic disease}

::Q154:: A three years old male preschooler presented with high-grade fever of 3 days duration with decreased playfulness. He had also reddish eyes, cough, and watery diarrhea for 2 days. There were also similar illnesses in the vicinity. What is the likely cause of the above disorder?

{~B.Pertussis

=Measles virus

~N.Meningitidis

~Plasmodium species}

::Q155:: A child with minimal change nephrotic syndrome has generalized edema. The child has been receiving Furosemide BID for 3 days. What is the next appropriate treatment plan to reduce edema?

{~Increasing the dose of Furosemide

~ Adding a second diuretic

= Giving IV albumin

~Eliminating all fluids and sodium from child's diet}

::Q156:: A child knows his age and sex, handedness identified, ride tricycle and stand momentarily with one foot and able to imitate cross. What additional development is expected from this child?

{~Skip

~Draw triangle

~Tell story

=Count three objects}

::Q157:: A 60 months old child came to well-child care with his mother. When you trace back his vaccination history, he never took any type of vaccine, otherwise he had no any indicator of immunocompromization. What would be your advice about vaccination?

{~Give one dose of pentavalent

=No need of catch up vaccine

~Give two dose pentavalent with 8wk interval

~Restart as new and continue the routine schedule}

::Q158:: A four years old male child from Metema presented with high grade fever of 3 days duration associated with chills, loss of appetite, and easy fatigability. Upon your evaluation he is lethargic, and febrile with a blood sugar of 30mg/dl. You decided to send a blood smear for Giemsa-stain. Of the following one is not true.

{~Thick smear is used to quickly scan large numbers of infected erythrocytes

~Thin smear is used for parasite species identification

=Thick smear is used to determine the parasitemia level

~Thin smear is used to assess for treatment response on follow-up}

::Q159:: In an evaluation of a child with suspected bacteremia, which one of the following is the single most important factor to maximize the yield from blood culture?

{~Obtaining two or more bottles

~Different sites sample collection

~Different time sample collection

=Obtaining a larger volume of blood}

::Q160:: A 19years old mother brought her 15days old male baby with a complaint of yellowish discoloration of the eye and constipation since birth associated with abdominal distension but no vomiting or failure to suck. Physical examination; icteric sclera, single transverse palmar crease, upslating palpebral fissure, and epicanthal fold. Laboratory: Bilirubin total- 12mg/dl with, TSH- 4μIU/L and Serum T4- 10μg/dL. Echocardiography showed 4mm defect at the membranous part of the ventricular septum. Which one of the following is the primary cause of this child's problem?

- {=Down syndrome
- ~Hypothyroidism
- ~Hyperbilirubinemia
- ~Aganglionic megacolon}

::Q161:: A 2 year old child presented to the pediatric outpatient clinic with bloody diarrhea crampy abdominal pain, and tenesmus for 02 days. Which of the following is the least likely diagnosis for the above case

- {=Giardiasis
- ~Amebiasis
- ~Intussusception
- ~Shigelosis}

::Q162:: A 10-year-old boy has a chronic cough. His mother notes that he coughs at night several times each week, enough to wake her but it does not seem to bother him. He has trouble keeping up with peers in sports activities and gym class; he says he runs out of breath easily and has to stop and rest. There is a family history of asthma in a paternal uncle, cystic fibrosis in a maternal cousin, and seasonal allergies in both parents. Physical examination findings are normal, including growth parameters and vital signs. Of the following, the BEST next step in his evaluation is to perform

- {~chest radiography
- ~ methacholine inhalation
- =spirometry
- ~sweat chloride testing}

::Q163:: A 6-month-old female infant is being evaluated at a health supervision visit. She has been exclusively breastfed and her parents have questions about the introduction of complementary foods. Both parents eat a strict vegetarian diet and plan to raise their daughter with the same dietary restrictions. Of the following, the vitamin MOST likely to be deficient in this infant's diet is

{=cobalamin

~niacin

~riboflavin

~thiamine}

::Q164:: Which of the following Pneumonia etiology correctly matched with its pathogenesis?

{=S. pneumoniae- local edema and proliferation

~Group A streptococcus- damages bronchial tree

~S.aureus- focal and lobar involvement

~M.pneumonia- Necrosis and cavitation of the lung parenchyma}

::Q165:: Which one of the following is correct about the management of Tonsillopharyngitis in children?

{~Mostly Surgical removal of recurrent Tonsillopharyngitis is recommended when the child is 1-2 years of age and it is highly effective

~The primary benefit and intent of treatment of bacterial tonsillopharyngitis in school age children is prevention of ARF and PSGN

~Antibiotics initiation is highly effective when it is started after 9 days of illness

=Single dose of IM benzathine penicillin is recommended and effective in a child having poor compliant for oral penicillin in case of recurrent Pharyngitis }

::Q166:: Which of the following clinical parameter is not supportive for diagnosis of pertussis in a child who present with sub-acute cough?

{=The presence of persistent fever and tachypnea with the cough

~The cough persisted for more than 2 weeks

~A long series of coughs between which there is little /no inspiratory effort

~History of similar illness in the household}

::Q167:: While you are working in a pediatric emergency a mother brought her 3year old male toddler with a history of watery diarrhea and vomiting for 4 days. Up on further history, he was having recently sunken eyeballs, and eagerness to drink but no change in the urine output. His anthropometry revealed MUAC of 11cm with $Wt/A < -3$ WHO Z score with normal vital signs, how do you proceed with the management of this patient?

{~Determine RBS and start with ORS 75ml/kg over 4 hours

~Start with 15ml/kg of normal saline with 5% dextrose over 1 hour

=Check for RBS and start 5ml/kg of ReSoMal every 30 minutes for 2 hours then hourly

~Insert nasogastric tube and give 50ml of 10% dextrose}

::Q168:: What is the next step of management in a 4-year-old child with severe shock who has a difficult intravenous access?

{~Insert central (jugular) venous line

~Venous cut down

=Intraosseous cannulation

~Rehydrate with NG tube with ORS}

::Q169:: A full-term large-for-gestational age neonate who is 4 hours of age is being evaluated in the normal newborn nursery. The infant was born via spontaneous vaginal delivery, vertex presentation, to a primigravida mother after 28 hours of labor. The neonate's vital signs are normal and his physical examination findings are remarkable only for a soft mass that covers the crown of the head. Of the following, this condition is MOST likely associated with

{=cranial molding

~falling hematocrit

~linear skull fracture

~low platelet count}

::Q170:: 10 minutes old macrosomic baby born from a mother with gestational DM referred from maternity for neonatal evaluation. Which of the following condition is not anticipated in this neonate?

{~Polycythemia

~Hypoglycemia

~Transient hypertrophic cardiomyopathy

=Subcutaneous fat necrosis}

::Q171:: Promotion of good health to improve quality of life through nutrition, the health professional recommend the community to follow food pyramid .Therefore, according to WHO, what is the recommended food source from the bottom to the tip of the pyramid?

{~Cereals and grain — ~~meat and meat product~~ ,and milk products — fruit and
vegetables — fats, oils and sweet foods
=Cereals and grain — fruit and vegetables — ~~meat and meat product~~ ,and milk
products — fats, oils and sweet foods
~Fruit and vegetables — Cereals and grain — ~~meat and meat product~~ ,and milk
products — fats, oils and sweet foods
~Fruit and vegetables — ~~meat and meat product~~ ,and milk products — Cereals and grain
fats, oils and sweet foods }

::Q172:: In District “X” there is high prevalence of Iodine deficiency disorder(IDD), then district health office discuss with the health professional to determine effective methods of nutritional intervention in terms of cost, easily acceptance by the community. Therefore as health professional which method is most likely effective?

{=Fortification

~Supplementation

~Nutrition education about Dietary diversity

~Early detection of the case and RX }

::Q173:: In internally displaced population (IDP), under five years children are vulnerable for nutritional problem. Therefore as health professional which one of the following nutritional indicator is used to classify wasting?

{~W/H Z-score $>-3SD$ and $\leq-2SD$

~W/A Z-score $\leq-3 SD$

~W/A Z-score $>-3SD$ and $\leq-2SD$

=W/H Z-score $<-2SD$ }

::Q174:: The ability of different information technology systems and software applications to communicate, to exchange data accurately, effectively and consistently is

{~Standardization

~Legalization

=Interoperability

~Normalization }

::Q175:: Which one of the following statements is wrong about DHIS2?

{~The purpose of DHIS2 is to provide a comprehensive platform for data collection, validation, reporting and analysis for decision making

~DHIS2 works either online or offline

~Data collection in DHIS2 is based on three dimensions

=All data must be entered into DHIS2 by hand}

::Q176:: Which one of the following statements is wrong about clinical decision support system?

{~Providing information electronically about protocols and standards for healthcare professionals to use in diagnosing and treating patients

~A clinical decision support system is enhancing medical decisions with targeted clinical knowledge, patient information, and other health information.

~The main purpose of CDSS is to provide timely information to clinicians, patients, and others to inform decisions about health care.

=It is a means of requesting diagnostic tests and treatments electronically and receiving the results }

::Q177:: According to the current health care tier system General hospitals are under a. Primary level

{~Primary level

=Secondary level

~Tertiary level

~Comprehensive level }

::Q178:: One of the following is not effective time management strategy

{~A. Goal setting

~Prioritization

=Procrastination

~Delegation }

::Q179:: Which one of the following could have broader scope among others?

{~Health Services

=Health Care

~C. Health Service Organizations

~D. Health center }

::Q180:: Which health insurance mechanism is more appropriate to Ethiopia to improve health services coverage for individuals working in the formal sectors:

{~Community insurance

~Private insurance

=Social health insurance

~All of the above }

::Q181:: The type of health services that has been provided to all citizens free of charge regardless of level of income is:

{~Fee waiver system
=Exempted service
~Health insurance
~Public goods}

::Q182:: A doctor of a given healthcare facility conducted a simple surgery for a patient at emergency room that have an inflammation on his body and collected the used sharps such as: syringes, needles and blades in waste container. What is the color and type of container used to collect waste in this case?

{~Red color plastic bin
~Red color safety box
~Black color plastic bin
=Yellow color safety box}

::Q183:: Mr. Abebe ate lunch at a famous restaurant and became ill because of the contaminated food he ate after 60 minutes. The possible disease-causing contaminated food he ate in this scenario is due to

{~Food infection
=Food intoxication
~Food adulteration
~Food toxico-infection}

::Q184:: Data compiled from weather monitoring stations showed that the annual average temperature increased dramatically, though monthly variations were apparent from the trend analysis. Scientists were interested to determine the factor responsible for the observed trend. What is the most likely cause in this case?

{~Depletion of the ozone layer
~Radioactive fallout
=Greenhouse effect
~Acid rain}

::Q185:: Water quality is one of the problems in a rural community that causes the spread of communicable diseases among under-five children in a developing country. A government starts to communicate with the NGO to solve the identified problems through community participation. After the pre-assessment of the community water sources, the stakeholder tries to design a community-based

water treatment strategy. From the above scenarios, one is the choice of water treatment strategies to reduce initially for solving the community's prior problems.

- {~Trace elements in the water
- ~Chemical and toxic substances in the water
- ~Cause of Hardness in the water
- =Pathogenic organisms in the water}

::Q186:: An organ dysfunction resulting from infection during pregnancy

- {~Maternal infection
- =Maternal sepsis
- ~Maternal near miss
- ~Post abortion infection}

::Q187:: Which one of the following is odd ?

- {~Haemorrhage
- ~Pregnancy-induced hypertension
- ~Infection
- =Anemia }

::Q188:: Why Safe Motherhood merits its place on the international agenda?

- {~The MM Ratio shows the greatest disparity between rich and poor countries
- ~Maternal death is a leading cause of adult female death in many countries
- ~Most importantly, maternal deaths are preventable
- =All }

::Q189:: In a certain community, there was a belief to hold materials made of iron by mothers during the postpartum period. This belief was proven neutral for the mother as well as for the whole community, as a health educator what will be your role in such situation?

- {~Educate the community to stop such behavior
- ~Encourage the mother to stop holding the material
- =Better to leave it as it is
- ~Punishment may be appropriate in such situation }

::Q190:: Which of the following is **false** statement about behavior?

{=It cannot be modified

~People becomes healthy or not healthy as a result of their behavior

~It is affected by peer pressure

~It may be both conscious & unconscious}

::Q191:: A patient was admitted to a psychiatric unit for suicidal ideation. Before initiating treatment, the treating psychiatrist explains the treatment procedure and obtains the patient's consent to start medication after discussing alternative treatments, risks, and benefits. Which ethical principle did the psychiatrist follow in doing the above?

{~Justice

=Autonomy

~Paternalism

~Beneficence }

::Q192:: Which of the following is **NOT** true about informed consent?

{~There are exceptions to informed consent

~Information can be withheld by the doctor in special circumstances if it is damaging to the patient

~Patients may waive the right to informed consent

=All medical situations, even emergencies, require informed consent.}

::Q193:: A patient with schizophrenia is brought to the emergency room in an acutely agitated state. He is administered haloperidol 5 mg intramuscularly. A few minutes later, the nurse reports that the patient's neck is twisted to one side and his eyes are rolled back. What is the MOST likely diagnosis?

{~Neuroleptic malignant syndrome

~Akathisia

=Dystonia

~Tardive dyskinesia}

::Q194:: A patient presents with a delusion about being poisoned that has been present for Five months. The patient has no hallucinations or other psychotic symptoms. There has been no major impact on the patient's daily functioning. The patient has no mood symptoms. The most likely diagnosis is:

{~Schizophrenia

~Schizophreniform disorder

=Delusional disorder

~Schizoaffective disorder}

::Q195:: A consultation-liaison psychiatrist is called to evaluate a patient who is in denial of a major illness. The most important obligation of the psychiatrist at the first evaluation is to:

{~Confront the denial forcefully

~Meet with the patient's family

=Make sure the patient has been informed about the illness and treatment

~Obtain Neuropsychological testing}

::Q196:: A patient enters your office. She is agitated, acts seductively, wears colorful clothes that are bizarre in appearance, has an excessive amount of makeup on, and vacillates between being entertaining, hyperexcited, and threatening. Based on this information, her most likely diagnosis is:

{~Brief psychotic disorder

=Bipolar disorder

~Delusional disorder

~Body dysmorphic disorder}

::Q197:: You are called to evaluate a 60-year-old man with a history of depression. His family reports that he has not been himself for the past 5days. On examination, he makes poor eye contact, is

inattentive, mutters incoherently, keeps rearranging pieces of paper on his bed tray with no apparent logic, and drifts off to sleep while you are talking to him. What is his most likely diagnosis?

- {~Brief Psychotic disorder
- ~Dementia
- =Delirium
- ~Obsessive-compulsive disorder}

::Q198:: A 45-year-old woman comes to the emergency room by ambulance unconscious and barely breathing. Paramedics found an empty bottle of 90 tablets of 2mg clonazepam on her dresser that was filled at the pharmacy the day before. One of the first agents to administer to this patient in the acute setting is:

- {~Naloxone
- =Flumazenil
- ~Atropine
- ~Epinephrine}

::Q199:: Which form of psychotherapy encourages the patient to speak “whatever comes to mind”?

- {~Cognitive behavioral therapy
- =Psychodynamic Psychotherapy
- ~Interpersonal therapy
- ~Supportive Psychotherapy}

::Q200:: When choosing medication for acute agitation, which of the following is most likely to produce symptoms of restlessness such as the need to be in constant motion, rocking from foot to foot, and pacing?

- {~Lorazepam IM
- =Haloperidol IM

~Olanzapine PO

~Chlorpromazine PO}